



DEPARTMENT OF REGULATORY AGENCIES

Colorado Podiatry Board

PODIATRY RULES AND REGULATIONS

3 CCR 712-1

[Editor's Notes follow the text of the rules at the end of this CCR Document.]

1.1 AUTHORITY

These regulations are adopted pursuant to the authority in sections 12-20-204 and 12-290-106(1)(a), C.R.S., and are intended to be consistent with the requirements of the State Administrative Procedures Act, sections 24-4-101, *et seq.* (the "APA"), C.R.S., and the Podiatry Practice Act, sections 12-290-101, *et seq.* (the "Practice Act"), C.R.S.

1.2 SCOPE AND PURPOSE

These regulations shall govern the process to become a licensed podiatrist and the practice of podiatry in Colorado.

1.3 APPLICABILITY

The provisions of these regulations shall be applicable to the practice of podiatry in Colorado.

1.4 DEFINITIONS

This Rule is promulgated pursuant to sections 12-20-204 and 12-290-106(1)(a), C.R.S.

- A. "Practice of Podiatry" is defined by section 12-290-102(3), C.R.S.
- B. "Mid-calf" means fifty-percent of the total distance between the talus and the tibial plateau.
- C. "Systemic Conditions" mean a condition that affects one or more parts or organs of the body other than those parts found in the human toe, foot, ankle, and/or tendons that insert into the foot.

1.5 CLARIFYING THE SCOPE OF THE PRACTICE OF PODIATRY IN COLORADO

This Rule is promulgated pursuant to sections 12-20-204, 12-290-102(3), and 12-290-106(1)(a), C.R.S.

- A. A podiatrist may possess, order, prescribe, dispense or administer preparations, medicines and/or drugs, including relevant braces, orthotics, prostheses and footwear, in order to treat conditions of the human toe, foot, ankle, and/or tendons that insert into the foot or in order to treat local manifestations of systemic conditions affecting the human toe, foot, ankle, and/or tendons that insert into the foot, except that a podiatrist may not administer an anesthetic other than a local anesthetic.

- B. Within the limitations set forth below, a podiatrist shall perform a patient history and physical examinations that may include complete examinations that are necessary for the effective identification and treatment of conditions of the human toe, foot, ankle, tendons that insert into the foot, and/or soft tissues would below the mid-calf. Complete examinations are often necessary for the identification and assessment of systemic and localized conditions contributing to or causing conditions of the human toe, foot, ankle, tendons that insert into the foot, and/or soft tissue wounds below the mid-calf. Such examinations may encompass the evaluation of all organ systems or body parts. However, as set forth in section 12-290-102(3)(c), C.R.S., and where generally accepted standards of podiatric practice so require, a podiatrist shall also assure that a licensed health care provider who is authorized by law to treat systemic conditions, also performs a history and physical examination for the patient.
- C. When treating soft tissue wounds below the mid-calf, a podiatrist shall conform to all requirements set forth in sections 12-290-102(3)(c) and 12-290-102(4), C.R.S., this Rule, and the generally accepted standards of podiatric practice.
- D. A podiatrist may amputate a portion of the foot but may not disarticulate between the talus and the tibia.
- E. A podiatrist may take emergency measures that are reasonable and necessary to protect and stabilize the patient's health until an appropriate provider authorized by law to treat the condition can intervene.
- F. A podiatrist performing a podiatric medical or surgical procedure on the human toe, foot, ankle, tendons that insert into the foot, and/or soft tissue wounds below the mid-calf, may perform more proximal measures that are incidental to the procedure and that are reasonable and necessary to perform the procedure in accordance with generally accepted standards of podiatric medicine.

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1.6 PODIATRY LICENSURE

This Rule is promulgated pursuant to sections 12-20-202, 12-20-204, 12-290-106(1)(a), 12-290-107, and 12-290-112, C.R.S.

A. INITIAL LICENSURE

An applicant for initial licensure must timely complete a Board approved application form establishing compliance with section 12-290-107, C.R.S., and the following requirements:

1. Graduation from a Board approved podiatry school;
2. Passage of the written PMLEXIS examination of the National Board of Podiatric Medical Examiners or any successor organization as determined by scores established by the Board;
3. Passage of the basic sciences examination of the National Board of Podiatric Medical Examiners or any successor organization;
4. Completion of one-year of residency as set forth in section 12-290-107(1)(c), C.R.S., as used in section (A) of this Rule, "an approved residency" is a residency of at least one year in a hospital conforming to the minimum standards of resident training established by the council on podiatric medical education or any successor organization.

5. Documentation that the applicant has been enrolled in podiatric medical school or residency or has been engaged in the active practice of podiatry during the two years immediately preceding the date of the current Colorado application, or documentation that the applicant has complied with the requirements of Section (A)(3) above within the two years immediately preceding the date of the current Colorado application.
6. As used in Section (A) of this Rule, "active practice of podiatry" means the applicant has engaged in the practice of podiatry at least twenty hours per week during the preceding two years with no more than a six month continuous absence from the practice of podiatry.
7. An applicant who cannot demonstrate continued competency by compliance with the above criteria may demonstrate competence by complying with other evaluation, education, training and/or monitoring the Board may require to establish continued competence. Such requirements shall be at the discretion of the Board.

B. LICENSURE BY ENDORSEMENT

In lieu of applying for an initial license to practice podiatry in Colorado, applicants who are licensed to practice podiatry in another jurisdiction, through the federal government, or who holds a military occupational specialty, as defined in section 24-4-201, C.R.S., may apply for licensure by endorsement pursuant to section 12-20-202(3), C.R.S. An applicant for licensure by endorsement must timely complete a Board approved application form establishing compliance with the following requirements:

1. Graduation from a Board approved podiatry school;
2. Passage (at the time of licensure in another jurisdiction) of the written PMLEXIS examination of the National Board of Podiatric Medical Examiners or any successor or predecessor organization as determined by scores established by the Board; except that the Board may, on a case-by-case basis, waive the requirement of this Section (B)(3) of this Rule for applicants for licensure by endorsement who have not passed the PMLEXIS examination, upon the provision of evidence satisfactory to the Board that the applicant's practice, experience, training, and/or education otherwise constitutes substantially equivalent qualifications.
3. Passage (at the time of licensure in another jurisdiction) of the basic sciences examination of the National Board of Podiatric Medical Examiners or any successor or predecessor organization, except that this requirement does not apply to applicants licensed by examination in another jurisdiction prior to 1970;
4. Compliance with training requirements comparable to those required by the Board (Colorado) at the time of the applicant's original licensure in another jurisdiction;
5. Possession of a license to practice podiatry in another jurisdiction that has not been revoked, suspended or subject to disciplinary or adverse actions; and
6. Documentation that the applicant has been engaged in the active practice of podiatry for the two years immediately preceding the date of the current Colorado application.
7. Submission of satisfactory proof that the applicant has held for at least one year a current and valid license to practice podiatry in another jurisdiction with a scope of practice that is substantially similar to the scope of practice for podiatrists as specified in Article 290 of Title 12, C.R.S., and these Rules.

8. As used in this Section (B) of this Rule, "active practice of podiatry" means the applicant has engaged in the practice of podiatry at least twenty hours per week during the preceding two years with no more than a six month continuous absence from the practice of podiatry; except that where appropriate for applicants for licensure by endorsement the Board may allow the applicant to fulfill the "active practice of podiatry" requirement by other means. It is anticipated that such exceptions shall be rare, and the decision as to what constitutes the active practice of podiatry shall be in the discretion of the Board.
9. An applicant who cannot demonstrate continued competency by compliance with the above criteria may demonstrate competence by complying with other evaluation, education, training and/or monitoring the Board may require to establish continued competence. Such requirements shall be at the discretion of the Board.

C. CREDIT FOR MILITARY, EDUCATION, TRAINING, OR EXPERIENCE

The purpose for the promulgation of this Rule is to satisfy the requirement of section 12-20-202(4), C.R.S., related to applying military education, training, and experience to qualifications for licensure.

1. An applicant for licensure may submit information about the applicant's education, training, or experience acquired during military service. It is the applicant's responsibility to provide timely and complete information for the Board's review.
2. In order to meet the requirements for licensure, such education, training, or experience must be substantially equivalent to the required qualifications that are otherwise applicable at the time the application is received by the Board.
3. The Board will determine, on a case-by-case basis, whether the applicant's military education, training, or experience meet the requirements for licensure.
4. Documentation of military experience, education, or training may include, but is not limited to, the applicant's Certificate of Release or Discharge from Active Duty (DD-214), Verification of Military Experience Training (DD-2586), military transcript, training records, evaluation reports, or letters from commanding officers describing the applicant's practice.

1.7 LICENSE RENEWAL PROCEDURES

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-290-119, C.R.S.

- A. The Board will not renew a license until the licensee has complied with the following requirements:
1. The licensee shall pay the Board a renewal fee to be determined and collected pursuant to section 12-20-105, C.R.S.;
 2. The licensee shall fully and accurately complete the Board's renewal questionnaire pursuant to section 12-290-119(2), C.R.S.;
 3. The licensee shall provide information that the licensee has complied with the financial responsibility requirements set forth in section 12-290-104(2), C.R.S., and Board Rule 1.13; and
 4. The licensee shall complete an attestation regarding continuing education pursuant to section 12-290-119, C.R.S., and Board Rule 1.12.

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- B. An applicant for license renewal, unless the applicant qualifies for an exemption, is required to fulfill the substance use prevention training requirements set forth in subsection (D) of Rule 1.12.
- C. Pursuant to section 12-20-202, C.R.S., licensees shall have a sixty-day grace period after the expiration of his or her license to renew such license without the imposition of a disciplinary sanction for practicing on an expired license. During this grace period a delinquency fee will be charged for late renewals.
- D. If a licensee fails to comply with the requirements listed above prior to the date on which the licensee is required to complete the renewal process including the grace period provided by section 12-20-202(1)(e), C.R.S., the license of such licensee shall lapse.
- E. At any point before, during or after the renewal process, a licensee's license may be subject to disciplinary action pursuant to sections 12-290-108 and 12-290-113, C.R.S. or as otherwise provided by the Article 290 of Title 12, C.R.S. Such disciplinary action may occur regardless of whether the licensee's license has lapsed.
- F. When a licensee's license lapses, the licensee may file a Board approved application for reinstatement with the Board. The licensee may be reinstated only upon compliance with the following conditions:
1. The licensee shall pay a reinstatement fee determined by the Board pursuant to section 12-20-105, C.R.S.;
 2. The licensee shall fully and accurately complete all portions of the Board's application for reinstatement;
 3. The licensee shall provide information that the licensee has complied with the financial responsibility requirements set forth in section 12-290-104(2), C.R.S., and Board Rule 1.13;
 4. The licensee shall complete an attestation regarding continuing education pursuant to section 12-290-119, C.R.S., and Board Rule 1.12;
 5. The licensee must comply with the requirements of sections 12-290-119 and 12-20-202(2)(c)(II), C.R.S., and Board Rule 1.8 regarding demonstration of continued competence; and
 6. If the licensee has any charges pending against the licensee's license, the Board may defer action on the pending application for reinstatement and proceed with disciplinary action as provided by section 12-290-113, C.R.S. Pursuant to any such disciplinary action, the Board will determine whether to reinstate with or without conditions or impose other sanctions as authorized by Article 290 of Title 12, C.R.S.

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1.8 LICENSE REINSTATEMENT AND THE DEMONSTRATION OF CONTINUED PROFESSIONAL COMPETENCE

This Rule is promulgated pursuant to sections 12-20-202(2), 12-20-204, 12-290-106(1)(a) and, 12-290-119, C.R.S.

A. REQUIREMENTS

1. As used in Article 290, Title 12, C.R.S., the terms lapsed, expired and delinquent are synonymous.
2. The Board shall not reinstate a license that has lapsed pursuant to section 12-290-119, C.R.S., or otherwise unless the licensee first accurately completes the application form approved by the Board and pays the application fees established by the Board.
3. The Board shall defer action on any application for reinstatement if it decides, either before or after the application has been made, to investigate a complaint against the applicant as provided in section 12-290-113, C.R.S., and if the Board decides to refer such complaint to the Office of the Attorney General for disciplinary proceedings by formal complaint, the Board shall defer final action on such reinstatement application until a hearing has been held in accordance with section 12-290-113, C.R.S., and the Board has issued its final order in such matter.
4. The Board shall not reinstate the license of any applicant for reinstatement whose license has lapsed for more than two years unless the applicant first demonstrates continued professional competence pursuant to the requirements set forth below. The applicant for reinstatement bears the burden of demonstrating his/her continued professional competence and must furnish to the Board evidence in support of the asserted continued professional competence. The Board may request additional information from an applicant for reinstatement.
5. The Board has established the following criteria for determining whether an applicant for reinstatement has demonstrated continued professional competence as required by sections 12-290-119(3) and 12-20-202, C.R.S. An applicant must meet all applicable criteria to establish continued professional competence.
 - a.. License lapsed two to five years. An applicant whose license has been lapsed for two to five years must demonstrate continued professional competence by:
 - (1) Documenting a current license to practice podiatry in another jurisdiction; and
 - (2) Documenting an active practice of podiatry in the jurisdiction that issued the current license since the Colorado license lapsed; and
 - (3) Submitting letters of endorsement from three licensed podiatrists or physicians who have observed the applicant's practice and who attest to the applicant's professional competence; and
 - (4) Submitting an attestation that the podiatrist has met the continuing education requirements set forth in Board policy.
 - b. License lapsed more than five years. An applicant whose license has been lapsed more than five years must demonstrate continued professional competence by:

- (1) Documenting a current license to practice podiatry in another jurisdiction; and
 - (2) Documenting an active practice of podiatry in the jurisdiction that issued the current license since the Colorado license lapsed; and
 - (3) Submitting letters of endorsement from three licensed podiatrists or physicians who have observed the applicant's practice and who attest to the applicant's professional competence; and
 - (4) Submitting an attestation that the podiatrist has met the continuing education requirements set forth in Board policy; and
 - (5) Demonstrating that the applicant has in the past passed the written PMLEXIS examination of the National Board of Podiatric Medical Examiners or any successor organization as determined by scores established by the Colorado Podiatry Board.
- c. Alternate method of establishing continued professional competence. Any podiatrist who does not demonstrate continued professional competence by compliance with the above criteria may demonstrate continued professional competency by:
 - (1) Passage within the preceding twenty-four months of the written PMLEXIS examination of the National Board of Podiatric Medical Examiners or any successor organization as determined by scores established by the Board; and/or
 - (2) Complying with any other evaluation, education, training and/or monitoring the Board may require to establish continued professional competence. Such requirements shall be at the discretion of the Board.
- B. As used in this Rule, "active practice of podiatry" means the applicant has engaged in the practice of podiatry at least twenty hours per week over the applicable time period with no more than a six month continuous absence from the practice of podiatry.
- C. An applicant for licensure reinstatement, unless the applicant qualifies for an exemption, is required to fulfill the substance use prevention training requirements set forth in subsection (D) of Rule 1.12.

1.9 THE MAINTENANCE OF CURRENT ADDRESS AND OTHER PERSONAL INFORMATION

This Rule is promulgated pursuant to sections 12-20-204 and 12-290-106(1)(a), C.R.S.

- A. A licensee's address for purposes of sending a "30-Day Letter" pursuant to section 12-290-113(2)(a), C.R.S., for purposes of issuing a formal complaint pursuant to section 12-290-113(3)(a), C.R.S., shall be the mailing address as indicated by the licensee on the application for initial licensure.

If a licensee provides an electronic mail "email" address, that email address shall be the licensee's address of record for all other Board purposes.

Licensees shall inform the Board in a clear, explicit and unambiguous written statement of any name, address, telephone or email change within thirty days of the change. Personal information may also be updated by the licensee via electronic means made available by the Board or by any other manner approved by the Board. The mere receipt of correspondence from a licensee showing a new address shall not be sufficient to change an address.

- B. Thereafter, the licensee's last address of record with the Board shall be the address as indicated in the request for the change.
- C. In no event will the Board accept a change of address request which requests the address be changed for some, but not all, communications. Also, in no event shall the Board change the address if a licensee indicates that Board correspondence shall be marked "confidential".

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1.10 PODIATRY RESIDENCY

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), 12-290-104(1), and 12-290-110, C.R.S.

- A. Residents or fellows training in an approved residency program shall not engage in the practice of podiatry unless licensed pursuant to sections 12-290-110, 12-290-108, or 12-290-112, C.R.S.
- B. Residents or fellows may not practice podiatry outside the parameters of their approved residency program or the parameters of their podiatry training license unless licensed to practice podiatry in this state pursuant to sections 12-290-108 or 12-290-112, C.R.S.
- C. Any resident or fellow engaging in the practice of podiatry without an active license issued pursuant to sections 12-290-110, 12-290-108, or 12-290-112, C.R.S., may be in violation of, and subject to penalties set forth in section 12-290-116, C.R.S.
- D. A "resident" or "fellow" for purposes of this rule means a person currently training in, or accepted into, an approved residency program for the purpose of podiatry training.
- E. An "approved residency" program for purposes of section 12-290-110, C.R.S., and this Rule shall meet one of the following criteria:
 - 1. Is hospital-based and approved by the Council of Podiatric Medical Education (CPME) of the American Podiatric Medical Association, or
 - 2. Is hospital-based and found, after investigation by the Colorado Podiatry Board, to provide residency training that is substantially equivalent to that provided by CPME-approved residency programs.
- F. An "approved podiatric medical school" for purposes of section 12-290-110, C.R.S., and this Rule means a school of podiatry at which not less than a two-year pre-podiatry course and a four-year course of podiatry is required and that is recognized and approved by the Colorado Podiatry Board.
- G. A "podiatry training license" for purpose of this Rule is a license issued pursuant to section 12-290-110, C.R.S.

1.11 COLLABORATIVE AGREEMENTS AND PRACTICE BY PHYSICIAN ASSISTANTS (PAs)

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-290-117(1) and (2), C.R.S. This rule governs the licensure and conduct of licensed physician assistants and not persons performing delegated podiatric tasks pursuant to section 12-290-116(6)(c), C.R.S.

A. REQUIREMENTS

In order to engage in practice as a physician assistant in collaboration with a licensed podiatrist pursuant to the provisions of section 12-290-117, C.R.S., a physician assistant must hold a current license to practice issued by the Colorado Medical Board.

B. EXTENT AND MANNER IN WHICH A PA MAY PERFORM TASKS CONSTITUTING THE PRACTICE OF PODIATRY IN COLLABORATION WITH A LICENSED PODIATRIST.

1. Identification of PAs:

The PA must be identified as a physician assistant to patients by wearing an identification plate upon their person setting forth their name and non-abbreviated title, and (ii) is not advertised or represented in any manner which would mislead patients or the public generally.

C. PRESCRIPTION AND DISPENSING OF DRUGS

1. A licensed physician assistant may issue a prescription order:

- a. The order is written, not verbal; and
- b. Each and every prescription and refill order is entered on the patient's chart; and
- c. Each written prescription order contains in printed form the name, address and telephone number of the health facility or specialty clinic where the PA is practicing.

2. All drugs dispensed or administered by PAs to third parties shall be prepackaged in a unit-of-use package and shall be labeled to show the name of the PA.

3. PAs shall not write or sign prescriptions or perform any services which the PA is not qualified or authorized to prescribe or perform.

D. REPORTING REQUIREMENTS

Each licensed collaborating podiatrist shall file with the Board information required on the forms provided by the Board within thirty days of the employment of such PA, and shall notify the Board of the termination of employment of any such collaborating persons previously registered within seven days of such termination. The filing of such information shall not be deemed as an approval by the Board of the job descriptions of such collaborating persons or the adequacy of their direction and collaboration by the licensed podiatrist. However, such information will be maintained by the Board for the purposes of (i) investigating complaints against licensed podiatrists collaborating with such PAs and (ii) maintaining a registry of the licensed podiatrists and PAs.

E. DISCIPLINE

Physician assistants are subject to the disciplinary procedures set forth in section 12-240-125, C.R.S., of the Medical Practice Act.

1.12 ONGOING PROFESSIONAL DEVELOPMENT

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), 12-290-119, and 12-30-114, C.R.S. These Rules were developed in conjunction with statewide professional associations that represent podiatrists.

A. RENEWAL

An application for the renewal of licensure to practice podiatry in Colorado shall include a demonstration by the applicant of their ongoing professional development. Licensees must attest at the time of renewal that they have obtained a minimum of ten hours of continuing education sufficient to maintain currency in the field of podiatry. This demonstration shall be evidenced by:

1. Participation in the continued competency program;
2. Participation in a program of ongoing professional development through an entity approved by the Board and deemed to have satisfied the requirements set forth in section 12-290-119(1)(b), C.R.S.; or
3. A licensed podiatrist serving in active military duty who meets the criteria established in section 12-20-302, C.R.S., and subsection (E) of these Rules may request from the Board an exemption from the ongoing professional development requirement for renewal or reinstatement.
4. Compliance with subsection (C) of this Rule regarding substance use prevention training.

B. CONTINUED COMPETENCY PROGRAM

The continuing competency ("CC") program shall consist of the following:

1. The completion of the reflective self-assessment tool on an annual basis.
2. The annual establishment of learning goals and the completion of ten hours of continuing education as required by Board Rule 1.12. The ten hours of continuing education should be undertaken to achieve a learning goal and based on the annual reflective self-assessment.
3. The completion of an assessment of knowledge and skill ("AKS") meeting Board criteria or approval once every five years. The five year cycle shall begin upon the renewal of a license or upon the first renewal or reinstatement of a license thereafter.
 - a. The AKS must meet the following criteria:
 - (1) Be scored;
 - (2) Be objective;
 - (3) Be administered by a reputable entity and drafted by a qualified podiatrist; and

- (4) Deliver to the candidate a certificate of completion or other documentation of completion stating the number of continuing medical education ("CME") hours credits earned, if any.
 - b. Administrative approval. The following AKS have been determined by the Board to meet the criteria established by the Board:
 - (1) The American Academy of Orthopedic Surgeons ("AAOS") orthopedic self-assessment examination (current version); or
 - (2) The Podiatry Insurance Company of America ("PICA") home study (current version).
 - c. If an AKS delivers CME credit, the podiatrist may count those credits toward the mandatory ten CME for the corresponding renewal cycle.
- 4. Documentation required. In order to satisfy an audit of compliance with the ongoing professional development requirement, a licensee must be able to provide to the Board all of the following:
 - a. Reflective self-assessment tool:

Submission of a signed and dated attestation of a reflective self-assessment tool.
 - b. Learning goals:
 - (1) Submission of certificates of completion of at least ten CME credits required by Board Rule 1.12 and this Rule, or
 - (2) Submission of a signed, completed and executed learning goal tool.
 - c. Submission of a certificate demonstrating the completion of an assessment of knowledge and skills that meets the criteria established by the Board.

C. DEEMED STATUS

- 1. Licensed podiatrists who are certified by the below listed institutions, and participate in that institution's maintenance of competency process, shall be deemed by the Board to have satisfied the ongoing professional development requirements set forth in section 12-290-119(1)(b), C.R.S., hereinafter known as "deemed status":
 - a. American Board of Foot and Ankle Surgery (ABFAS)
 - b. American Board of Podiatric Medicine (ABPM)
- 2. In order to renew a license, the deemed status must be demonstrated to the Board by utilizing the online continuing competency portal or submission of paper compliance forms.
- 3. Documentation required. In order to satisfy an audit of compliance with the ongoing professional development requirement claiming deemed status, a licensee must be able to provide the Board:
 - a. Proof of active membership from an institution mentioned above, or

- b. Other documentation approved by the Board.
- 4. Deemed status does not supersede the renewal, reinstatement, or reactivation of licensure requirements outlined in section 12-290-119(1)(a), C.R.S., Board Rule 1.12, and subsection (D) of Rule 1.12.
- D. **SUBSTANCE USE PREVENTION TRAINING FOR LICENSE RENEWAL, REACTIVATION, OR REINSTATEMENT**
 - 1. Pursuant to section 12-30-114, C.R.S., every podiatrist, except those exempted under subsection (D)(3), is required to complete at least one hour of training per renewal period in order to demonstrate competency regarding the topics/areas specified in section 12-30-114(1)(a), C.R.S.
 - 2. Training, for the purposes of this section includes, but is not limited to, relevant continuing education courses; self-study of relevant scholarly articles or relevant policies/guidelines; peer review proceedings that involve opioid prescribing; relevant volunteer service; attendance at a relevant conference (or portion of a conference); teaching a relevant class/course; or participation in a relevant presentation, such as with your practice. All such training must cover or be related to the topics specified in section 12-30-114(1)(a), C.R.S.
 - 3. The Board shall exempt a podiatrist from the requirements of this section who qualifies for either exemption set forth in section 12-30-114(1)(b), C.R.S.
 - 4. This section shall apply to any applicant for reinstatement or reactivation of an expired or inactive license pursuant to section 12-20-203, C.R.S. and, as applicable, Rule 1.8.
 - 5. Applicants for license renewal, reactivation, or reinstatement shall attest during the application process to either their compliance with this substance use training requirement or their qualifying for an exemption, as specified in subsection (D)(3) of this Rule.
 - 6. The Board may audit compliance with this section. Podiatrists should be prepared to submit documentation of their compliance with this substance use training requirement or their qualification for an exemption, upon request by the Board.
 - 7. This training for substance use prevention is in addition to the required hours for continued competency and these substance use prevention hours may not be applied towards any required hours for continued competency. Completed substance use prevention training hours that meet the requirements for continuing education, as specified in Rule 110, may be applied towards the minimum hours required in Rule 110.
- E. **MILITARY EXEMPTION**
 - 1. Pursuant to section 12-20-302, C.R.S., podiatrists who have been called to federally funded active duty for more than 120 days for the purpose of serving in a war, emergency, or contingency, may request an exemption from the ongoing professional development requirements for the renewal or reinstatement of a license for the renewal cycle that falls within the period of service or within six months following the completion of service.
 - 2. In order to renew a license, the military exemption must be demonstrated to the Board utilizing the online continuing competency portal or submission of paper compliance forms.

3. Documentation required. In order to satisfy an audit of compliance with the ongoing professional development requirement claiming military exemption, the licensee must be able to provide the Board:

Written documents that evidence satisfaction of the criteria established in section 12-20-302, C.R.S.

F. DISCOVERABILITY

Records, documents, and information obtained by the Board to ensure compliance with this Rule are exempted from the provisions of any law requiring that they be open to public inspection, please reference section 12-290-113(10), C.R.S.

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1.13 FINANCIAL RESPONSIBILITY STANDARDS

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-290-104(2), C.R.S.

- A. Pursuant to the requirements of section 12-290-104(2), C.R.S., every podiatrist who performs surgical procedures as part of his podiatric practice and who holds or desires to obtain a Colorado license must maintain professional liability insurance coverage with an insurance company authorized to do business in this state in a minimum indemnity amount of \$1,000,000 per incident, and \$3,000,000 annual aggregate per year.
- B. Pursuant to these rules, every podiatrist who does not perform surgical procedures and who holds or desires to obtain a Colorado license must maintain professional liability insurance coverage with an insurance company authorized to do business in this state in a minimum indemnity amount of \$500,000 per incident, and \$1,000,000 annual aggregate per year.
- C. Pursuant to these rules, a podiatrist whose podiatric practice falls entirely within one or more of the following categories is exempt from the requirements set forth in paragraphs (A) and (B), above:
 1. A podiatrist who is completely and permanently retired from the practice of podiatry.
 2. A podiatrist who does not engage in any patient care whatsoever within the State of Colorado.
 3. A federal civilian or military podiatrist whose practice is limited solely to that required by his federal/military agency.
 4. A podiatrist who is not engaged in any patient care whatsoever (administrators, researchers, academicians, those engaged in endeavors outside of podiatry, e.g.).
 5. A podiatrist who is covered by individual commercial professional liability coverage maintained by an employer/contracting agency in the amounts set forth in paragraph A and B, above.
 6. A podiatrist whose practice is as a public employee under the "Colorado Governmental Immunity Act."
- D. Any podiatrist who claims exemption from the financial responsibility requirements must provide such information as may be requested by the Board in order to establish eligibility for any such exemption.

- E. For purposes of this Rule, “surgical procedures” means any procedure that involves cutting through the skin to a level deeper than the dermis.

1.14 MISLEADING, DECEPTIVE OR FALSE ADVERTISING

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-290-108(3)(k), C.R.S. This Rule applies to advertising in all types of media including, but not limited to, print, radio, television and the Internet.

- A. Podiatrists should take special care to advertise truthfully and avoid exploitation of their position of trust. A podiatrist must avoid misleading the public because of the potential consequences of misinformation regarding health care and the importance of the interests affected by the choice of a podiatrist. Podiatrists are responsible for the contents of their own advertisements and should review such advertisements to assure adherence to ethical standards.

Therefore, podiatrists shall avoid the following types of advertising:

1. Claims that the services performed, personnel employed, and/or materials or office equipment used are professionally superior to that which is ordinarily performed, employed, and/or used, or that convey the message that one licensee is better than another unless superiority of services, personnel, materials or equipment can be substantiated;
 2. The misleading use of a claim regarding board certification, or of an unearned or non-health degree in any advertisement that is likely to cause confusion or misunderstanding as to the credentials, education, or licensure of a health care professional;
 3. Advertising that has the effect of intimidating or exerting undue pressure;
 4. Advertising that uses unsubstantiated testimonials;
 5. Advertising that creates an unjustified expectation or guarantees satisfaction or a cure;
 6. Advertising that offers gratuitous services or discounts, the purpose of which is to deceive the public, or
 7. Advertising that is otherwise misleading, deceptive or false.
- B. At the time any type of advertisement is placed, the podiatrist must possess and rely upon information that, when produced, would substantiate the truthfulness of any assertion, omission or claim set forth in the advertisement. When using a subjective testimonial whose truthfulness cannot be substantiated, the advertisement should also include disclaimers or warnings as to the credentials of the person making the testimonial.

Adopted: MARCH 4, 2005, EFFECTIVE MAY 1, 2005; REVISED 6/3/22; EFFECTIVE 7/30/22;

1.15 EDUCATION AND TRAINING STANDARDS FOR UNLICENSED PERSONNEL EXPOSING IONIZING RADIATION

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-290-202(1)(a), C.R.S. These rules establish minimum educational and training standards to be met by all unlicensed persons prior to administration of radiation; however, the Board encourages education and training beyond this minimum.

A. RULES AND REGULATIONS REGARDING EDUCATION AND TRAINING STANDARDS FOR UNLICENSED PERSONNEL EXPOSING IONIZING RADIATION

No podiatric physician nor podiatrist shall allow any unlicensed person to operate a machine source of ionizing radiation or to administer such radiation to any patient unless such unlicensed person has met the standards set forth herein.

1. All unlicensed podiatric personnel exposing ionizing radiation must:
 - a. Be a minimum of eighteen years of age; and
 - b. Successfully complete minimum safety education and training for operating machine sources of ionizing radiation and administering such radiation to patients.
2. Such education and training shall consist of at least eight hours of educational instruction or supervised training and shall include, at a minimum, education or training in each of the following areas:
 - a. Podiatric nomenclature;
 - b. Machine operation exposure factor;
 - c. Operator and patient safety;
 - d. Practical or clinical experience in:
 1. Foot and ankle techniques for exposing radiographs;
 2. Film handling and storage;
 3. Processing procedures;
 4. Patient record documentation for radiographs.
3. Written verification of education and training shall be provided by the sponsoring agency, educational institution, podiatrist, or licensee to each participant upon completion. This written verification shall be co-signed by the unlicensed person. A copy of the verification shall be maintained in each unlicensed person's employment record located at the employment site, and the original shall be maintained by the unlicensed person. Written verification of completion of education and training must include:
 - a. The name of the agency, educational institution, or licensee who provided the education and training;
 - b. Verification of the hours of educational instruction and supervised training provided;
 - c. The date of completion of the education and training; and
 - d. A description of the exposure techniques for which education and training have been provided; i.e., foot and ankle radiographs.

4. Education and training may be obtained from programs approved by the appropriate authority of any state. The appropriate authority may be a private occupational school approved pursuant to sections 23-64-101, *et seq.*, C.R.S., the State Board of Community Colleges and Occupational Education, or the equivalent in any other state, or as approved by the Colorado Podiatry Board. Such programs shall include the education and training as specified in subsection B, above.
5. The required education and training may be provided on the job by a licensed podiatrist, or by a podiatric medical assistant who has at least six months of experience and who has completed the required education and training. Such education and training provided on the job must comply with subsection (B) above.
6. For the purposes of this Rule, an x-ray technician who has satisfactorily completed a course of instruction and has been certified by the American Registry of Radiological Technologists shall be deemed to have met the training requirements established in subsection (B), above.

1.16 REQUIRED DISCLOSURE TO PATIENTS – CONVICTION OF OR DISCIPLINE BASED ON SEXUAL MISCONDUCT

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-30-115, C.R.S.

- A. A provider, shall disclose to a patient, as defined in section 12-30-115(1)(a), C.R.S., instances of sexual misconduct, including a conviction or guilty plea as set forth in section 12-30-115(2)(a), C.R.S., or final agency action resulting in probation or limitation of the provider's ability to practice as set forth in section 12-30-115(2)(b), C.R.S.
- B. Form of Disclosure: The written disclosure shall include all information specified in section 12-30-115(3), C.R.S., and consistent with the sample model disclosure form as set forth in Appendix A to these Rules. The patient must, through his or her signature on the disclosure form, acknowledge the receipt of the disclosure and agree to treatment with the provider.
- C. Timing of Disclosure: This disclosure shall be provided to a patient the same day the patient schedules a professional services appointment with the provider. If an appointment is scheduled the same day that services will be provided or if an appointment is not necessary, the disclosure must be provided in advance of the treatment.
 1. The written disclosure and agreement to treatment must be completed prior to each treatment appointment with a patient unless the treatment will occur in a series over multiple appointments or a patient schedules follow-up treatment appointments.
 2. For treatment series or follow-up treatment appointments, one disclosure prior to the first appointment is sufficient, unless the information the provider is required to disclose pursuant to section 12-30-115, C.R.S., has changed since the most recent disclosure, in which case an updated disclosure must be provided to a patient and signed before treatment may continue.
- D. As set forth in section 12-30-115(3)(e), C.R.S., the requirement to disclose the conviction, guilty plea, or agency action ends when the provider has satisfied the requirements of the probation or other limitation and is no longer on probation or otherwise subject to a limitation on the ability to practice the provider's profession.
- E. A provider need not make the disclosure required by this Rule before providing professional services to the patient if any of the following applies as set forth in section 12-30-115(4), C.R.S.:

1. The patient is unconscious or otherwise unable to comprehend the disclosure and sign an acknowledgment of receipt of the disclosure pursuant to section 12-30-115(3)(d), C.R.S., and a guardian of the patient is unavailable to comprehend the disclosure and sign the acknowledgement;
 2. The visit occurs in an emergency room or freestanding emergency department or the visit is unscheduled, including consultations in inpatient facilities; or
 3. The provider who will be treating the patient during the visit is not known to the patient until immediately prior to the start of the visit.
- F. licensee provider who does not have a direct treatment relationship or have direct contact with the patient is not required to make the disclosure required by this Rule.

1.17 ELECTRONIC PRESCRIBING OF CONTROLLED SUBSTANCES

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-30-111(2), C.R.S.

- A. Pursuant to section 12-30-111(1)(a), C.R.S., a prescriber shall prescribe a controlled substance as set forth in section 12-30-111(1)(a), C.R.S., only by electronic prescription transmitted to a pharmacy unless an exception in section 12-30-111(1)(a), C.R.S., applies.
- B. A “temporary technological failure,” for purposes of section 12-30-111(1)(a)(I), C.R.S., is when:
1. A necessary prescribing software program is inaccessible or otherwise not operational;
 2. Required technology fails to start; or
 3. During a period when a virus or cyber security breach is actively putting patient data and transmission at risk.
- C. A “temporary electrical failure,” for purposes of section 12-30-111(1)(a)(I), C.R.S., is a short-term loss of electrical power at the place of business that lasts no more than forty-eight hours or two consecutive business days unless there is a showing of undue hardship.
- D. An “economic hardship,” for purposes of section 12-30-111(1)(a)(XI), C.R.S., is a measurement of relative need taking into consideration the individual gross receipts and net profits, cost of compliance, and type of software upgrade required. In order for a prescriber to demonstrate economic hardship, the prescriber must submit to the Board for a final determination:
1. A written statement explaining the economic hardship, including supporting documentation to demonstrate economic hardship. Supporting documentation may include the most recent tax return or other business records that show gross receipts and net profits. The Board reserves the right to request additional documentation to support the request, if necessary. The request must also include the requested duration of the economic hardship.
 2. If the Board determines there should be an economic hardship exception for the prescriber, then the Board will determine the duration of the economic hardship exception, which shall not exceed one year from the date the exception was granted.

3. In order to renew a request for an economic hardship exception, the prescriber must submit a request to renew the exception in writing to the Board no less than two months prior to the expiration of the economic hardship exception. The prescriber must provide a written statement explaining the need to renew the economic hardship, including supporting documentation.

1.18 RULES REGARDING THE USE OF BENZODIAZEPINE

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 12-30-109(6), C.R.S.

- A. Licensees must limit any prescription for a continuous benzodiazepine to a 30-day supply, for any patient who has not been prescribed a benzodiazepine in the last 12 months.

Prior to prescribing the second fill of a benzodiazepine for a condition that is not exempt under section 12-280-404(4)(a.5) C.R.S., a licensee must comply with the requirements of section 12-280-404(4), C.R.S.

- B. The limitation stated in Section (A) of this Rule does not apply to patients for whom licensees prescribe benzodiazepines for the following conditions:

1. Epilepsy;
2. A seizure, a seizure disorder, or a suspected seizure disorder;
3. Spasticity;
4. Alcohol withdrawal; or
5. A neurological condition, including a post-traumatic brain injury or catatonia.

- C. These rules do not require or encourage abrupt discontinuation, limitation, or withdrawal of benzodiazepines. Licensees are expected to follow generally accepted standards of podiatry practice, based on an individual patient's needs, in tapering benzodiazepine prescriptions.

1.19 COLORADO PODIATRY BOARD RULES AND REGULATIONS FOR DECLARATORY ORDERS

This Rule is promulgated pursuant to sections 12-20-204, 12-290-106(1)(a), and 24-4-105(11), C.R.S.

- A. Any person may petition the Board for a declaratory order to terminate controversies or to remove uncertainties as to the applicability to the petitioner of any statutory provision or of any rule or order of the Board.
- B. The Board will determine, in its discretion and without notice to petitioner, whether to rule upon any such petition. If the Board determines that it will not rule upon such a petition, the Board shall promptly notify the petitioner of its decision. Any of the following grounds, among others, may be sufficient reason to refuse to entertain a petition.
 1. Failure to comply with Section C of this Rule.
 2. A ruling on the petition will not terminate the controversy nor remove uncertainties as to the applicability to petitioner of any statutory provision or rule or order of the Board.

3. The petition Involves any subject, question or issue which is the subject of, or is involved in, a matter (including a hearing, investigation or complaint) currently pending before the Board, particularly, but not limited to, any such matter directly involving the petitioner.
 4. The petitioner seeks a ruling on a moot or hypothetical question, or will result in an advisory ruling or opinion, having no direct applicability to the petitioner.
 5. Petitioner has some other adequate legal remedy, other than an action for declaratory relief pursuant to C.R.C.P. 57, which will terminate the controversy or remove any uncertainty concerning applicability of the statute, rule or order in question.
- C. Any petition filed pursuant to this Rule shall set forth the following:
1. The name and address of the petitioner; whether the petitioner is licensed by the Colorado Podiatry Board.
 2. The statute, rule or order to which the petitioner relates.
 3. A concise statement of all of the facts necessary to show the nature of the controversy or uncertainty and the manner in which the statute, rule or order in question applies or potentially applies to the petitioner. Petitioner may also include a concise statement of the legal authorities upon which petitioner relies.
 4. A concise statement of the specific declaratory order sought by petitioner.
- D. If the Board determines that it will rule on the petition, the following procedures shall apply:
1. The Board may rule upon the petition without holding an evidentiary hearing.
In such case:
 - a. Any ruling of the Board will apply only to the extent of the facts presented in the petition and in any clarifying information submitted in writing to the Board.
 - b. The Board may order the petitioner to file a written clarification of factual matters, a written brief, memorandum or statement of position.
 - c. The Board may set the petition, upon due notice to petitioner, for a non-evidentiary hearing.
 - d. The Board may dispose of the petition on the sole basis of the matters set forth in the petition.
 - e. The Board may take administrative notice of commonly known facts within its expertise or contained in its records and consider such facts in its disposition of the petition.
 - f. If the Board rules upon the petition without a hearing, it shall promptly notify the petitioner of its decision.

2. The Board may, in its discretion, set the petition for an evidentiary hearing, conducted in conformance with section 24-4-105, C.R.S., upon due notice to petitioner, for the purpose of obtaining additional facts or information or to determine the truth of any facts set forth in the petition. The notice to the petitioner setting such hearing shall set forth, to the extent known, the factual or other matters into which the Board intends to inquire. For the purpose of such a hearing, the petitioner shall have the burden of proving all of the facts stated in the petition, all of the facts necessary to show the nature of the controversy or uncertainty and the manner in which the statute, rule or order in question applies or potentially applies to the petitioner and any other facts the petitioner desires to consider.
- E. The parties to any proceeding pursuant to this Rule shall be the Board and the petitioner. Any other person may seek leave of the Board to intervene will be granted at the sole discretion of the Board. A petition to intervene shall set forth the same matters as required by Section C of this Rule of this chapter. Any reference to a "petitioner" in this Rule also refers to any person who has been granted leave to intervene by the Board.

1.20 CONCERNING HEALTH CARE PROVIDER DISCLOSURES TO CONSUMERS ABOUT THE POTENTIAL EFFECTS OF RECEIVING EMERGENCY OR NONEMERGENCY SERVICES FROM AN OUT-OF-NETWORK PROVIDER

This rule is promulgated pursuant to sections 12-20-204, 12-30-112, and 12-290-106(1)(a), C.R.S., in consultation with the Commissioner of Insurance and the State Board of Health. The purpose of this rule is to establish requirements for health care providers to provide disclosures to covered persons who are utilizing a health benefit plan about the potential of balance billing when receiving post-stabilization services or covered non-emergency services from an out-of-network provider at an in-network facility. This rule applies to health care providers. Balance billing by a health care provider is only permitted when the criteria established in Colorado law, including but not limited to sections 12-30-112 and 12-30-113, C.R.S., are met.

- A. Definitions, for purposes of this rule, are as follows:
1. "Ancillary Services" as defined in section 12-30-112(1)(a), C.R.S.
 2. "Balance Bill" and "Balance Billing" as defined in section 10-16-704(19)(c), C.R.S.
 3. "Covered Non-emergency Services" means services that are not emergency services as defined in section 10-16-704(19)(e), C.R.S., are services covered by a covered person's health benefit plan, and are not ancillary services as defined in section 12-30-112(1)(a), C.R.S.
 4. "Covered Person" as defined by section 10-16-102(15), C.R.S.
 5. "Health Benefit Plan" as defined by section 10-16-102(32), C.R.S.
 6. "Health Care Provider" means "provider," as defined in section 10-16-102(56), C.R.S.
 7. "In-Network Facility" means a facility, either within or outside of Colorado, that, under a contract with a carrier or with its contractor or subcontractor, has agreed to provide health-care services to covered persons with an expectation of receiving payment, other than coinsurance, copayments, or deductibles, directly or indirectly, from the carrier.
 8. "Out-of-Network Provider" means a Health Care Provider who is not a "Participating Provider."
 9. "Participating Provider" as defined in section 10-16-102(46), C.R.S.

10. "Post-Stabilization Services" means covered services related to an emergency medical condition, as defined in section 10-16-704(19)(d), C.R.S., that are provided once the criteria set forth in section 10-16-704(19)(e)(III) are met.
- B. Disclosure requirements.
1. An Out-of-Network Provider may balance bill a Covered Person for (a) Post-Stabilization Services in accordance with section 10-16-704, C.R.S., and (b) Covered Non-Emergency Services provided in an In-Network Facility that are not Ancillary Services, but only if the Out-of-Network Provider meets the requirements set forth in section 12-30-112(3.5), C.R.S.
 2. If a Covered Person may incur a claim for Post-Stabilization Services or Covered Non-Emergency Services from an Out-of-Network Provider, the Out-of-Network Provider shall complete and provide the notice contained in Appendix "B" to these rules or a similar disclosure which complies with the requirements set forth in section 12-30-112(3.5), C.R.S.
 3. Such notice must be provided in the 15 most common languages in Colorado, which, for purposes of this regulation, are English, Spanish, Vietnamese, Chinese, Korean, Russian, Amharic, Arabic, German, French, Nepali, Tagalog, Japanese, Cushite, Persian.
- C. If applicable and in addition to their responsibilities under this Rule, Health Care Providers shall also comply with the "No Surprises Act," 42 U.S.C.A. § 300gg-111, Pub.L 116-260, as amended.
- D. Noncompliance with this Rule may result in the imposition of any of discipline made available by section 12-290-108(3)(g), C.R.S.

APPENDIX A

MODEL SEXUAL MISCONDUCT DISCLOSURE STATEMENT

DISCLAIMER: This Model Sexual Misconduct Disclosure Statement is to be used as a guide only and is aimed only to assist the provider in complying with section 12-30-115, C.R.S., and the rules promulgated pursuant to this statute by the Director. As a licensed, registered, and/or certified health care licensee in the State of Colorado, you are responsible for ensuring that you are in compliance with state statutes and rules. While the information below must be included in your Sexual Misconduct Disclosure Statement pursuant to section 12-30-115, C.R.S., you are welcome to include additional information that specifically applies to your situation and practice.

- A. Provider information, including, at a minimum: name, business address, and business telephone number.
- B. A listing of any final convictions of or a guilty plea to a sex offense, as defined in section 16-11.7-102(3), C.R.S.
- C. For each such conviction or guilty plea, the provider shall provide, at a minimum:
 - 1. The date that the final judgment of conviction or guilty plea was entered;
 - 2. The nature of the offense or conduct that led to the final conviction or guilty plea;
 - 3. The type, scope, and duration of the sentence or other penalty imposed, including whether:
 - a. The provider entered a guilty plea or was convicted pursuant to a criminal adjudication;
 - b. The provider was placed on probation and, if so, the duration and terms of the probation and the date the probation ends; and
 - c. The jurisdiction that imposed the final conviction or issued an order approving the guilty plea.
- D. A listing of any final agency action by a professional regulatory board or agency that results in probationary status or other limitation on the provider's ability to practice if the final agency action is based in whole or in part on:
 - 1. a conviction for or a guilty plea to a sex offense, as defined in section 16-11.7-102(3), C.R.S., or a finding by the professional regulatory board or Director that the provider committed a sex offense, as defined in as defined in section 16-11.7-102(3), C.R.S.; or
 - 2. a finding by a professional regulatory board or agency that the provider engaged in unprofessional conduct or other conduct that is grounds for discipline under the part or article of Title 12 of the Colorado Revised Statutes that regulates the provider's profession, where the failure or conduct is related to, includes, or involves sexual misconduct that results in harm to a patient or presents a significant risk of public harm to patients.
- E. For each such final agency action by a professional regulatory board or agency the provider shall provide, at a minimum:

1. The type, scope, and duration of the agency action imposed, including whether:
 - a. the regulator and provider entered into a stipulation;
 - b. the agency action resulted from an adjudicated decision;
 - c. the provider was placed on probation and, if so, the duration and terms of probation; and
 - d. the professional regulatory board or agency imposed any limitations on the provider's practice and, if so, a description of the specific limitations and the duration of the limitations.
2. The nature of the offense or conduct, including the grounds for probation or practice limitations specified in the final agency action;
3. The date the final agency action was issued;
4. The date the probation status or practice limitation ends; and
5. The contact information for the professional regulatory board or agency that imposed the final agency action on the provider, including information on how to file a complaint.

Sample Signature Block

I have received and read the sexual misconduct disclosure by [Provider Name] and I agree to treatment by [Provider Name].

Print Patient Name

Patient or Responsible Party's Signature

Date

If signed by Responsible Party (parent, legal guardian, or custodian), print Responsible Party's name and relationship to patient:

Print Responsible Party Name

Print Relationship to Patient

Provider Signature

Date

APPENDIX B

BALANCE BILLING NOTICE

PATIENT RIGHTS INFORMATION

Check the appropriate box:

- ☐ Your provider is proposing to use an out-of-network care provider in delivering your health care service(s). This facility is in-network with your insurance but there may be care providers involved in your care that are out-of-network.
- ☐ Your provider is proposing to deliver post-stabilization care at an out-of-network facility. You have received emergency services at the out-of-network facility and are now stabilized, but you may require additional health care services.

You are not required to consent to receive these services from the out-of-network care provider or continue to receive post-stabilization care at an out-of-network facility. If you choose to proceed with the proposed out-of-network care provider or facility you may be billed for costs detailed in the Good Faith Estimate below. The additional costs you pay may not accrue toward insurance cost sharing or deductibles.

You may choose to use an in-network provider from the list below or you may choose to transfer your care to an in-network facility for post-stabilization services. If you choose to proceed with an in-network provider or transfer to an in-network facility, the cost will not exceed the amount allowed by your insurance plan.

You chose to receive this Notice ☐ electronically or ☐ in paper form.

This notice must have been provided to you, either in paper or electronically, per your preference within the following timeframes:

1. At least seventy-two hours in advance of the date of services, if the appointment was scheduled at least seventy-two hours in advance;
2. At least three hours before the scheduled appointment, if the appointment was made less than seventy-two hours in advance.

This is not a contract for services. Your provider is required to retain this form for seven years.

This form must be available to you in the 15 languages most common to the geographic region where your provider is located, which include English, Spanish, Vietnamese, Chinese, Korean, Russian, Amharic, Arabic, German, French, Nepali, Tagalog, Japanese, Cushite, and Persian.

BILLING ADVISEMENT

(choose applicable billing scenario)

- ☐ Out-of-Network Provider at In-Network Facility

Your provider is proposing to use an out-of-network care provider in delivering your service(s). That out-of-network provider is/are:

[PROVIDER NAME]

Description of service(s) to be provided by an in-network facility by an out-of-network provider:

[SERVICE]

You scheduled the service(s) on [DATE] at [TIME]. You are planning to receive the service(s) stated above on [DATE] at [TIME]

Do you need prior authorization from your insurance company for the service(s) provided at this facility?

[Y / N]

Good Faith Estimate for the total cost of the service(s) to you, the patient:

[\$]

Does this facility employ in-network care providers who provide the service(s) detailed above?

[Y / N]

If Yes, the in-network care provider(s) who provide the service(s) are:

[PROVIDER NAME]

NOTE: If there is no in-network provider to provide the service(s) at this in-network facility you cannot be balanced billed for the services provided by the out-of-network provider. **OR**

☐ Post-Stabilization Services

Your provider is proposing to deliver post-stabilization care at an out-of-network facility. The out-of-network facility is and/or the provider(s) is/are:

[FACILITY/PROVIDER NAME]

Description of post-stabilization service(s) to be provided by an out-of-network facility or provider:

[SERVICE]

You scheduled the service(s) on [DATE] at [TIME]. You are planning to receive the service(s) stated above on [DATE] at [TIME].

Good Faith Estimate for the total cost of the service(s) to you, the patient:

[\$]

I [PATIENT NAME] received this form at [TIME] on [DATE].

SIGNATURE OF PATIENT

[TIME] and [DATE]

Editor's Notes

History

Entire rule eff. 01/30/2009.

Entire rule eff. 02/03/2010.

Entire rule emer. rule eff. 07/01/2010.

Entire rule eff. 10/30/2010.

Entire rule eff. 08/14/2019.

Entire rule eff. 07/30/2022.

Rules 1.20, 1.21 emer. rules eff. 09/09/2022; expired 01/07/2023.

Rules 1.22, 1.23 emer. rules eff. 10/26/2022.

Rules 1.22, 1.23 emer. rules eff. 11/11/2022.

Rules 1.22, 1.23 emer. rules eff. 01/09/2023; expired 05/09/2023.

Rules 1.6 B, 1.11 B-C, 1.18, 1.20, 1.21, 1.22, Appendix B eff. 01/30/2023.

Rule 1.11 emer. rule eff. 09/08/2023.

Rule 1.11 eff. 12/30/2023. Rules 1.20, 1.21 repealed eff. 12/30/2023. Rule 1.22 renumbered as 1.20 eff. 12/30/2023.

Rules 1.20, Appendix B eff. 10/30/2025.

Annotations

Rules 1.21 B. and 1.21 C. were to be expired by Senate Bill 23-102. However, these rules were not adopted on or after November 1, 2021 and before November 1, 2022 pursuant to section 24-4-103(8)(c), C.R.S., and therefore were not removed.